

Domestic abuse evidence

16-letter demo

Eight GP letters and eight domestic-abuse support-service letters, using varied prose rather than template edits. Letters include apparently complete evidence, omissions and genuine ambiguity. The same model generated the letters and reviewed them in separate calls without seeing the intended labels; this is not independent expert validation.

All letters, people and organisations are fictional. Proposed labels are for demonstration and expert review, not actual LAA decisions. Labels address the stated evidence category, not overall eligibility for legal aid. Authenticity and professional registration have not been verified.

Generated with gpt-6.1-sol at medium reasoning via azure. Guidance: version 15, 15 June 2026. Prepared 3 October 2026.

Proposed label totals: 8 appears complete, 5 incomplete, 3 needs human review

Each sample is a separate fictional case. Similar names do not indicate linked records. This short pack pairs each letter with a proposed outcome. Full criterion-by-criterion evidence quotes and guidance references are in the detailed review and labelled JSONL.

The letters-only Word file omits the proposed outcomes. These are draft demo labels for expert feedback, not measured accuracy or predictions of actual LAA decisions.

Source: [official guidance PDF, version 15, 15 June 2026](#)

ID	Category	Proposed label
GP01	GP	Appears complete
GP02	GP	Incomplete
GP03	GP	Needs human review
GP04	GP	Appears complete
GP05	GP	Appears complete
GP06	GP	Needs human review
GP07	GP	Incomplete
GP08	GP	Appears complete
DA01	Support service	Appears complete
DA02	Support service	Needs human review
DA03	Support service	Appears complete
DA04	Support service	Incomplete
DA05	Support service	Appears complete
DA06	Support service	Incomplete
DA07	Support service	Incomplete
DA08	Support service	Appears complete

GP01 | GP letter — Schedule 1, paragraph 11

Merewillow Family Practice

24 September 2026

Dear Ms Brindle,

Re: Lena Orwick

I am writing at Ms Orwick's request, with her consent, to summarise my assessment of her health.

On 9 September 2026 I examined Ms Orwick by telephone during a booked clinical consultation. We discussed her sleep, mood and ability to manage ordinary activities, and I assessed her symptoms and mental state. She was experiencing persistent anxiety, difficulty falling asleep and frequent waking. Her concentration had also deteriorated, making routine tasks unusually demanding.

My assessment was of an anxiety condition with significant sleep disturbance. Taking the clinical history and presentation together, my professional judgement is that this condition is consistent with the effects of domestic abuse. This is my clinical opinion, rather than simply a record of an explanation offered by the patient.

We agreed a treatment plan and arranged a further appointment to review her sleep and anxiety. She has remained engaged with her care despite finding conversations about her circumstances difficult.

I am a general practitioner registered with the General Medical Council and licensed to practise. My registration and licence were current at the consultation and remain current at the date of this letter.

Yours sincerely,

Dr Amara Vennick
General Practitioner

Proposed outcome: Appears complete

The letter identifies the applicant, confirms a clinical telephone examination, and gives the GP's own judgement that the assessed condition is consistent with domestic abuse.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP02 | GP letter — Schedule 1, paragraph 11

Alderfen Medical Rooms

Dr Cerys Halvern, General Practitioner

Registered with the General Medical Council; current licence to practise

18 August 2026

Dear Sir or Madam,

Re: Beatrice Nolling

Ms Nolling has asked for a letter recording the concerns she recently brought to the practice's attention. She has consented to the information below being shared with her solicitor.

On 12 August our reception team received a written message from Ms Nolling through the patient messaging system. In it, she described feeling increasingly anxious, sleeping poorly and struggling to keep up with everyday responsibilities. She also described restrictions on her access to household money and repeated demands to account for where she had been.

I have read that message and understand why she wishes to obtain advice about her family circumstances. The difficulties she describes deserve a sensitive response, particularly as she says that arranging private time to speak can be challenging.

The practice has offered her a choice of appointment times and explained how she can ask for correspondence to be handled discreetly. Her preferred arrangements for receiving letters have been noted on her record.

I hope this account of her communication with the practice is helpful. It records the information she supplied and the practical steps we have taken in response.

Yours faithfully,

Dr Cerys Halvern

General Practitioner

Proposed outcome: Incomplete

The letter records a patient message and administrative responses, but does not confirm a clinical examination or a professional judgement that an examined condition is consistent with domestic abuse.

GP3 (missing): Receiving and reading a written patient message does not confirm an examination in person, by telephone or by video.

GP4 (missing): The letter records the patient's account but gives no clinician's consistency judgement about injuries or a condition identified at examination.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP03 | GP letter — Schedule 1, paragraph 11

Thistlemere Surgery

6 July 2026

Dear Mr Fenwick,

Re: Nadia Pelloran

Further to your request, and with Ms Pelloran's permission, I have looked back at the entry concerning our telephone contact on 25 June. This followed her request for a call about sleep difficulties and arrangements for her next appointment.

During the call she mentioned recurrent nightmares, episodes of panic and difficulty concentrating at work. The note also records that she was staying elsewhere following a period of intimidation and controlling behaviour within her relationship. We discussed a quieter time for the practice to contact her and the availability of a follow-up appointment.

In my professional judgement, the anxiety and disturbed sleep recorded on that occasion are consistent with a condition arising in the context of domestic abuse. The symptoms were having a considerable effect on her day-to-day functioning.

I have recommended that her next appointment allow sufficient time to discuss treatment options.

I am a GP with current General Medical Council registration and a licence to practise, both also held on 25 June.

Yours sincerely,

Dr Ivo Rennick

Dictated and signed

Proposed outcome: Needs human review

The letter identifies the applicant and supplies a professional consistency judgement, but it is ambiguous whether the telephone contact constituted a clinical examination rather than symptom reporting and appointment arrangements.

GP3 (unclear): Telephone contact and symptom discussion are established, but it is unclear whether the GP clinically assessed the symptoms during that call or merely recorded reports and arranged further contact.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP04 | GP letter — Schedule 1, paragraph 11

Brackenfold Health Practice

30 September 2026

Private correspondence

Re: Tomas Wetherell

Consultation

I examined Mr Wetherell by video on 22 September 2026. The appointment included a clinical history, visual assessment of the affected areas and questions about pain and movement. The picture and sound were adequate for that assessment.

Findings

There was visible bruising over his upper arm and mild swelling around one wrist. He described tenderness when using that hand. Movement was preserved, although uncomfortable. I also identified marked anxiety and disrupted sleep. We discussed care for the wrist and the circumstances in which he should seek an in-person review.

Clinical opinion

The findings, considered alongside the history obtained during the consultation, fit with injuries and anxiety associated with domestic abuse. This is my medical assessment of his presentation; I cannot determine the precise mechanism of each injury from a video consultation.

I provide this summary with Mr Wetherell's consent. I am a general practitioner registered with the General Medical Council and licensed to practise. I held that registration and licence when I examined him and continue to hold both today.

Dr Elin Marrick

General Practitioner

Proposed outcome: Appears complete

The letter confirms a clinical video examination, describes identified injuries and symptoms, and expresses the examining GP's medical judgement that the findings fit with domestic abuse.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP05 | GP letter — Schedule 1, paragraph 11

Brackenmere Family Practice
24 September 2026

Dear Ms Fenwick,

Re: Tessa Marlow

You asked whether we could provide a medical account of Ms Marlow's earlier attendance. With her written consent, I have reviewed the consultation entry and subsequent treatment records held by this practice. I did not conduct the original examination.

Ms Marlow was examined in person on 18 November 2022 by my colleague, Dr Elin Voss, a general practitioner who was registered with the General Medical Council and licensed to practise at that time. Dr Voss remains registered and licensed, but is currently on extended leave and unavailable to prepare this letter. I am also a GMC-registered general practitioner with a licence to practise, held both at the time of that consultation and today.

The entry describes persistent disturbed sleep, heightened anxiety and difficulty concentrating. Dr Voss recorded a clinical assessment of an anxiety condition, rather than simply reproducing the account Ms Marlow gave. Her recorded professional judgement was that the condition and its presentation were consistent with Ms Marlow having experienced domestic abuse within her former relationship. No physical injury was recorded at that appointment.

The later notes show that sleep and day-to-day functioning improved during treatment, although progress was uneven. That improvement does not alter the findings made at the earlier examination. Having read the clinical record myself, I consider Dr Voss's assessment well supported by the symptoms and observations she documented.

I have confined this account to the medical findings relevant to your request; it is not intended as a complete history of Ms Marlow's care.

Yours sincerely,

Dr Rowan Pell
General Practitioner

Proposed outcome: Appears complete

The letter supports all applicable requirements, including record access and the original examiner's unavailability.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP06 | GP letter — Schedule 1, paragraph 11

Alder Quay Surgery
9 July 2026

Dear Mr Cresswell,

Re: Nerys Bellan

I am writing with Ms Bellan's permission concerning her consultation at this surgery on 21 May 2026. I am a general practitioner, registered with the General Medical Council and licensed to practise, and held that status at the time of her attendance.

The consultation was a clinical examination by video with Dr Mira Hesk, another GMC-registered and licensed GP. Dr Hesk was registered and licensed when she examined Ms Bellan. She is now on leave and unavailable to write personally.

A summary passed to me following the consultation describes recurrent panic episodes, interrupted sleep and marked apprehension. Dr Hesk wrote the consultation notes. The assessment recorded there was that Ms Bellan had an anxiety condition whose presentation was consistent with her having been a victim of domestic abuse. This was Dr Hesk's clinical judgement following the examination, rather than solely Ms Bellan's explanation of her symptoms.

Ms Bellan has since arranged a follow-up appointment to discuss treatment and the effect of the symptoms on her working day. This letter concerns the findings at the May consultation and does not attempt to predict how long her symptoms will continue.

Yours sincerely,

Dr Jonas Vale
General Practitioner

Proposed outcome: Needs human review

The professional status, video examination and clinical consistency judgement are supported. The different author's access to the medical records is ambiguous.

GP5 (unclear): The examiner's unavailability is explicit. However, receiving a summary and describing the notes does not clearly establish whether the author had access to the applicant's medical records.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP07 | GP letter — Schedule 1, paragraph 11

Willowglass Medical Practice
16 June 2026

Dear Ms Linton,

Re: Orla Daven

Orla asked me for a letter after our appointment last Thursday, 11 June. I examined her in person that afternoon. I am her GP and am registered with the General Medical Council with a current licence to practise; I also held registration and a licence at the appointment.

She has been having a difficult time with sleep, often waking very early and then feeling exhausted by lunchtime. During the consultation I assessed persistent insomnia, with fatigue and some loss of concentration. We talked through her evening routine, caffeine intake and the practical difficulty of resting when she has an early shift the next morning.

Orla believes that domestic abuse during her previous relationship explains the sleep problem. She told me that she still finds herself going over conversations from that period when the house is quiet. She wanted that part of her account included here, as she feels it is easily lost when appointments focus on sleep advice.

We agreed on a short sleep diary and a further appointment in three weeks. I suggested that she bring the diary even if it has gaps; it need not become another task to worry about.

With best wishes,

Dr Celia Norrick
General Practitioner

Proposed outcome: Incomplete

The letter confirms an examination and an insomnia assessment, but gives only the patient's attribution to domestic abuse, not the clinician's consistency judgement.

GP4 (missing): A condition is identified, but the only connection to domestic abuse is the patient's belief. The clinician does not express a professional judgement that the condition is consistent with domestic abuse.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

GP08 | GP letter — Schedule 1, paragraph 11

Merewick House Surgery
12 September 2026

Dear Sir or Madam,

Re: Mrs Ada Wrenfold

At Mrs Wrenfold's request, I confirm that I examined her in person at this surgery on 7 February 2024. I was then, and remain, a medical practitioner registered with the General Medical Council and licensed to practise as a general practitioner.

At that examination she was suffering from an anxiety condition, with poor sleep, episodes of panic and difficulty managing ordinary daily activities. My clinical judgement was that the condition identified at the consultation was consistent with her having been a victim of domestic abuse. This assessment took account of both the history obtained and her presentation during the examination.

She attended for several subsequent appointments and recieved treatment. By the autumn of 2024 her symptoms had substantially improved. I mention this to distinguish her condition at the original examination from her later progress.

The above account is drawn from my examination and the contemporaneous notes I made. Mrs Wrenfold has consented to its release.

Yours faithfully,

Dr Edwin Sallow
General Practitioner

Proposed outcome: Appears complete

The letter clearly supports the applicable professional-status, identification, examination and clinical-judgement requirements.

Basis: Schedule 1(11), guidance 2.52–2.60 and checklist, pp.18–21.

DA01 | Support-service letter — Schedule 1, paragraph 17

From: Isla Fenwick <isla.fenwick@rowanharbour.example>

To: Family Team <family@brackenfoldlaw.example>

Date: 18 September 2026

Subject: Support provided to Mairi Cresswell

Dear Family Team,

Mairi Cresswell has asked me to write about her contact with Rowan Harbour Support. We are a domestic abuse support organisation based in Scotland, United Kingdom, and have operated continuously since February 2014. I am Mairi's allocated support worker and have met with her five times since June.

Mairi described how her former partner expected her to account for each journey outside the house, repeatedly checked her messages and accused her of being disloyal when she spoke to friends. Since separation, she has received repeated demands to explain where she is and who is with her. During one appointment, she showed me a series of messages demanding photographs to prove her whereabouts. She had stopped attending an evening class because she feared the response afterwards.

Taking these conversations and the messages together, my professional judgement is that Mairi has experienced psychological domestic abuse and remains at risk of further controlling behaviour from her former partner.

We have worked through a safety plan, changed the privacy settings on her accounts together and helped her identify a safe way to keep important documents. I also supported her to reconnect with a trusted friend and accompanied her to a housing advice appointment.

She needed this support because the monitoring had left her isolated and unsure how to make ordinary decisions safely. Our work has focused on restoring practical choices without increasing that risk. She continues to attend fortnightly appointments.

Kind regards,

Isla Fenwick

Domestic Abuse Support Worker

Rowan Harbour Support

Proposed outcome: Appears complete

The email substantively supports all nine requirements for this demo category.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA02 | Support-service letter — Schedule 1, paragraph 17

Celyn Bridge Domestic Abuse Support
Wales, United Kingdom

22 July 2026

Dear Ms Aldercombe,

I am writing at the request of Elin Marrick, whom I have supported since April. Celyn Bridge began its domestic abuse work in Wales in 2013. There was a temporary closure during building repairs in 2021, before we reopened at our present premises the following spring.

Elin first came to us after leaving the home she shared with her husband. She described being required to hand over her wages, having to ask for money for bus fares and being challenged over small purchases. In our appointments she showed me messages in which he demanded receipts and threatened to cancel her access to their account. She also brought bank statements showing that her wages went into an account she could no longer access independently.

In my professional judgement, Elin has been subjected to domestic abuse involving financial control and intimidation. That view rests on our detailed discussions and the material she shared, rather than simply the fact that she sought help.

I helped Elin prepare a safe budget, supported her through opening an account in her own name and arranged food vouchers when she had no money available. We also completed a safety plan for collecting essential belongings.

She needed this assistance because she lacked access to everyday funds and was frightened that taking steps towards independence would lead to further threats. These were immediate difficulties, not merely concerns about future expenditure.

Yours sincerely,
Bethan Veylock
Senior Support Practitioner

Proposed outcome: Needs human review

The substantive abuse judgement and support requirements are supported, but uninterrupted organisational operation for at least six months is ambiguous.

S3 (unclear): Reopening occurred more than six months before the letter, but uninterrupted organisational operation during a qualifying subsequent period is not clearly confirmed.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA03 | Support-service letter — Schedule 1, paragraph 17

Alderwake Domestic Abuse Partnership
England, United Kingdom
1 October 2026

Dear Sir or Madam,

Re: Daniel Norcombe

Alderwake Domestic Abuse Partnership has operated without interruption in England since May 2019. Our neighbourhood outreach service opened on 1 August 2026. I joined the organisation in September and have personally held three support appointments with Daniel Norcombe through that service.

Daniel sought help concerning his former civil partner. In reaching my professional judgement, I have relied particularly on the following:

- Daniel's account of being repeatedly prevented from sleeping by demands that he answer questions about supposed infidelity.
- Messages he showed me threatening to disclose private information to his employer unless he returned home.
- His description of abandoning visits to his sister because each visit prompted prolonged accusations and demands for access to his phone.

Considered together, these matters demonstrate a pattern of coercive and psychological domestic abuse. In my reasonable professional judgement, Daniel has experienced that abuse and remains at risk of it continuing after separation.

The support I have already provided includes:

- Developing a safety plan covering unwanted visits and safe communication.
- Helping Daniel secure his email account and remove shared access to his calendar.
- Accompanying him to a housing appointment and helping him organise documents for that meeting.

Daniel needed this help because the threats made him fearful of seeking accommodation independently, while shared account access exposed his appointments and movements. He was also struggling to identify which communications required a response. Our sessions have given him practical ways to reduce those risks while maintaining necessary arrangements.

Yours faithfully,
Rory Penhalwick
Outreach Support Worker

Proposed outcome: Appears complete

All applicable requirements are supported. The newer outreach service and author's recent appointment do not undermine the organisation's confirmed continuity.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA04 | Support-service letter — Schedule 1, paragraph 17

Merehaven Domestic Abuse Centre
England, United Kingdom

14 August 2026

Dear Ms Wrenholt,

Re: Nadia Bellmere

I write with Nadia Bellmere's consent to confirm the support she has received from Merehaven Domestic Abuse Centre. We are based in England and provide specialist domestic abuse advocacy and practical support. I am the caseworker responsible for her support.

Across four appointments, Nadia described her husband taking her house keys during disagreements, preventing her from attending appointments and threatening to remove her belongings if she stayed with relatives. She showed me messages telling her she would be locked out if she did not return immediately. During our second meeting, she explained that she had missed a housing appointment because she could not leave the property with a key.

In my professional judgement, Nadia is experiencing domestic abuse through intimidation and control of her movements and access to her home.

I have completed a safety plan with her, helped her arrange secure storage for copies of essential documents and accompanied her to a rescheduled housing appointment. We have also agreed a safe method for our service to contact her.

Nadia required this support because she feared losing access to her home and belongings, and the restrictions were obstructing her attempts to obtain independent advice. Our assistance addressed those practical barriers while taking account of her safety.

Yours sincerely,
Serena Ashcombe
Domestic Abuse Caseworker

Proposed outcome: Incomplete

The letter supports the abuse judgement, its specific basis and completed support, but omits confirmation of at least six months' uninterrupted organisational operation.

S3 (missing): There is no statement of organisational operating duration or uninterrupted operation for at least six months.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA05 | Support-service letter — Schedule 1, paragraph 17

Bracken Lantern Domestic Abuse Service
York, England, United Kingdom
18 September 2026

Dear Sir or Madam,

I am writing at the request of Mr Ellis Norcombe, whom I have supported since June. Ellis initially found it difficult to describe his experiences as abuse because there had been no physical assault. That did not lessen the effect on his everyday life.

During our appointments he explained that his former cohabiting partner, Rowan Pellin, required him to send photographs showing where he was and repeatedly contacted his workplace when he did not answer immediately. I have seen messages demanding those photographs and threatening to tell his employer private information about him. Ellis also described abandoning visits to his sister because he expected prolonged questioning afterwards. These accounts were consistent across our meetings.

We have worked together on a practical safety plan, including safer arrangements for collecting belongings. I helped Ellis review his phone's location-sharing settings and accompanied him to an appointment with an independent housing adviser. He needed this assistance because he was still sharing digital accounts with Rowan, was uncertain how to leave those arrangements safely and had nowhere settled to stay after leaving their home.

Bracken Lantern provides specialist domestic abuse support from our base in York, in the United Kingdom. The organisation has operated continuously, without interruption, since February 2021.

Having assessed Ellis's account alongside the messages he showed me, my professional judgement is that he has experienced domestic abuse through monitoring, intimidation and control by his former partner. Our support remains available while he establishes independent arrangements.

Yours faithfully,

Merrin Dovale
Domestic Abuse Support Practitioner

Proposed outcome: Appears complete

The letter substantively supports all applicable paragraph 17 requirements.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA06 | Support-service letter — Schedule 1, paragraph 17

From: celia.varn@alderquay-support.example
To: intake@fenwickmere-law.example
Date: 22 September 2026
Subject: Support confirmation — Nessa Halver

Dear Ms Brell,

I write with Nessa Halver's consent to confirm her involvement with Alder Quay Support Partnership. I am the senior practitioner responsible for her case and have reviewed our records for the purpose of this letter.

The Partnership is based in Cardiff, Wales, United Kingdom, and provides specialist support to people experiencing domestic abuse. It has operated without interruption since November 2019.

Nessa was referred to us in July 2026 and has attended individual appointments during August and September. My assessment draws on the referral information, her testimony during those appointments and subsequent case discussions with our safeguarding lead. Having considered that material together, it is my reasonable professional judgement that Nessa has been a victim of domestic abuse within her former intimate relationship.

We have provided individual emotional support, completed a personal safety plan with her and helped her identify suitable sources of housing and welfare advice. With her permission, I also arranged an appointment with an independent advice service and supported her preparation for that appointment.

This assistance was needed because Nessa was distressed, unsure how to assess her current safety and finding it difficult to navigate services without support. Our involvement has given her a consistent point of contact while she considers her next steps. She remains on my active caseload.

Yours sincerely,

Celia Varn
Senior Domestic Abuse Practitioner
Alder Quay Support Partnership

Proposed outcome: Incomplete

The letter lacks specific matters supporting the author's professional judgement.

S6 (missing): Only generic evidence sources are listed. No specific conduct, incidents or other substantive matters underlying the abuse judgement are described.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA07 | Support-service letter — Schedule 1, paragraph 17

Mosslark Family Safety Centre
Bristol, England, United Kingdom
9 September 2026

Dear Ms Trenn,

Thank you for asking me to write about Iona Bellwick. I am her allocated domestic abuse worker at Mosslark. We are a specialist domestic abuse support organisation based in Bristol, in the United Kingdom, and have been operating continuously since April 2022.

Iona described her former partner waiting outside her workplace twice after she had asked him not to contact her. She showed me messages saying that he would keep turning up until she agreed to meet. One included a photograph of the staff entrance. Her account and those messages form the basis of my professional opinion that she is at risk of domestic abuse from her former partner.

Our work with Iona centres on safety planning and housing advocacy. The safety plan is to cover her journey home, a trusted contact and what to do if he appears again. On the advocacy side, I would liaise with her housing officer about safer access arrangements and keeping her new address confidential. These are the practical priorities in her current support plan.

Iona needs this help because she is worried about being approached alone and is unsure which housing arrangements can be changed. She has been particularly concerned about leaving work after dark.

Best wishes,

Tamsin Orvel
Domestic Abuse Worker

Proposed outcome: Incomplete

The letter describes planned support but does not establish what support was actually provided.

S7 (missing): The described measures are planned or conditional. The letter does not describe support actually delivered for her domestic-abuse-related needs.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.

DA08 | Support-service letter — Schedule 1, paragraph 17

Sablebridge Safety Project
Sheffield, England, United Kingdom
30 September 2026

To whom it may concern,

Re: Mara Vennick

Sablebridge Safety Project provides domestic abuse support in Sheffield, England, United Kingdom. The organisation began operating on 30 January 2026 and has operated continuously, without interruption, for eight months.

Mara Vennick attended our service on 7 and 14 September. She sought help following threats from her former partner, Tobin Ruskell, with whom she previously lived.

I read messages in which he said he would come to her new home, force his way inside and make her regret leaving. A further message referred to the street where she now lives, although she had not given him that address. Mara explained that she had changed her usual route home because she feared being followed.

In my reasonable professional judgement, Mara is at risk of domestic abuse from her former partner. She reported no physical injury; my assessment rests on the threats, his apparent knowledge of her location and the circumstances she described.

We completed a safety plan, agreed a check-in arrangement with a trusted friend and helped Mara secure her online accounts. With her consent, I contacted her landlord to request a review of entrance security.

She needed this practical support because her address appeared to have been discovered and she did not know how to reduce the resulting risks while remaining in her accommodation.

Yours faithfully,

Jalen Mervet
Service Coordinator

Proposed outcome: Appears complete

The letter supports all applicable requirements, including uninterrupted organisational operation and actual practical support.

Basis: Schedule 1(17), guidance 2.74–2.77 and checklist, pp.25–26.